



OFFICIAL

NMHS MHS PROCEDURE

Inpatient Hospital Campus Access and Community Outings

Scope (Audience)	All NMHS Mental Health Services (MHS) Inpatient Staff
Scope (Area)	All NMHS MHS Inpatient Areas (excluding State Forensic Mental Health Service)
Summary	Provides staff with information in relation to process, documentation requirements, site-specific areas for inpatient escorted ground access and unescorted ground access on hospital grounds and in the community.
Risk Rating	Medium
NSQHS Standards	Clinical Governance (Std 1) Partnering with Consumers (Std 2)
Policy Sponsor	Director of Medical Services MHSDHS
Policy Contact	Inpatient Operational Co Directors MHSDHS
Version	V2.0
Effective Date	19 November 2025
Review Date	19 November 2028

COMPLIANCE WITH THIS DOCUMENT IS MANDATORY

Do you have any *feedback*? Tell us [here](#) or scan the QR code to access the Policy Document Evaluation Form. For any *queries* about this document, please email the Document Contact listed above.



1. Aim

North Metropolitan Health Service Mental Health Services (NMHS MHS) will ensure patients in secure and locked wards have access to the rest of the hospital campus or community access, as deemed appropriate by the treating team.

Staffing requirements where patients require an escort will be determined in accordance with the [MH Clinical Risk Assessment and Management Policy](#), Department of Health (DoH) [MP 0012/16 Mental Health Consumers Who Are Missing or Absent Without Leave Policy](#), DoH [MP 00122/19 Clinical Incident Management Policy](#) and the DoH [Clinical Incident Management Guideline 2019](#).

2. Risk

Non-compliance with this procedure protocol will impact on the following risk categories:

Culture	☒	Organisational Development	☐
Emergency Management	☒	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

3. Key Points

There are two (2) types of hospital ground access:

- Escorted Ground Access (**EGA**)
- Unescorted Ground Access (**UGA**)

Ground access, either escorted or unescorted, does not involve leaving the hospital grounds where specified. Any permitted leave from the hospital grounds is considered “*leave*” or “*community access*” and not ground access. Sir Charles Gardiner Hospital (SCGH) does not have designated grounds, and as such, leave from SCGH is classified as community access.

The purpose of patient leave will be documented primarily on the form (PMR 11B) in the progress notes by the Medical Officer and ground access form. Any specific conditions or considerations must also be recorded to inform an individual plan for the patient, for example if the patient must be breathalysed on return a Progressive Risk Assessment (PRA) should be completed for new leave and any changes to leave.

For Osborne Park Older Adult (OA), EGA is authorised by medical staff, in consultation with the treating or Duty Consultant. A PRA is completed by staff. EGA is also documented in the progress notes, PSOLIS and Digital Medical Record (DMR).

Smoking is **not** an acceptable reason for ground access. Nicotine dependence will be assessed on admission by medical or nursing staff and prior to grounds access being initiated. If a patient commences smoking on EGA, the escort may stop EGA immediately and remove themselves from the hazard. An incident report should be completed for any personal exposure to smoke. Please refer to the [NMHS Smoke Free Policy](#), NMHS [MHS Smoke Free Procedure](#) and the [MHS Smoking Care Guideline](#).

EGA can precede UGA as part of a transition plan from a secure ward to an open ward, if appropriate.

At Digital Medical Record (DMR) sites, document leave in the PMR 11B (**Appendix 2**) primarily, then patient's progress notes. The PMR 11B should be placed in the work in progress file (WIP) for all patients. This provides an easy point of reference for staff to access the patient's recent agreed plan regarding leave.

In the event that nursing staff cancel a patient's leave while awaiting medical review, the leave on the Form 11B should be crossed out and initialled with a corresponding entry in the DMR.

All leave and any conditions must be reflected in the patient's PSOLIS treatment support and discharge plan (TSDP) and leave sticker (**Appendix 1**) (non DMR sites)/ PMR 11B Mental Health Approved Leave Form (DMR sites, if form used) and PRA template for Older Adult. Details should include a review date to ensure ongoing review.

Medical, Nursing and Allied Health staff will advise patients of their responsibilities when on ground or community access and ensure they have the relevant phone numbers of the ward/hospital.

If at any time, including during EGA or UGA, ward staff are concerned that the patient's risk profile has significantly deteriorated, they must immediately suspend further leave and request prompt medical review.

Prior to the initiation or change of leave, a PRA will be completed by a member of the medical staff, in consultation with the treating or Duty Consultant. For sites using DMR refer to local procedures for how to create the PRA in the DMR using a template.

Prior to the commencement of any type of leave the PMR11B, where utilised must be completed by the staff member who is allowing exit from the ward or is taking the patient on leave documenting their description.

Aboriginal Mental Health Liaison Service (AMHLS) staff can escort patients where appropriate.

There must be an assessment of patient's mental state prior to using any periods of leave.

Any period of leave will be documented in the ward report and progress notes.

The Shift Co-ordinator and delegated Clinical Observation Nurse will be aware of any patient going on or returning from leave as well as any changes made with the treating team to the conditions of leave.

All patients will be present and/or accounted for at the time of any shift handover where possible. Ground access during shift handover should be avoided where possible. This may be unavoidable with community access, there should be clear communication documented regarding expected time of return.

3.1 Escorted Ground Access

EGA is not appropriate for patients with a high risk of absconding or current risk factors relating to suicide or aggression.

- The first period of EGA may only be authorised by medical staff, in consultation with the treating or Duty Consultant.
- Ground access may be granted following consultation with nursing staff, and prior to being discussed with the patient.

- A PRA completed by staff in consultation with treating or Duty Consultant must be placed in the progress notes on granting a patient EGA. A patient may not leave the ward for the first period of EGA without this occurring. For sites using Digital Medical Record (DMR) refer to local procedures for how to create the PRA in the DMR using a template
- If EGA is approved for a patient, the frequency and timing of this will be dependent on staff availability. This will be determined by the Shift Co-ordinator. The patient should be given a time frame during which EGA is likely to occur and advised of the duration and frequency.
- EGA with the patient's carer or an appropriate adult may only be undertaken with the approval of the medical staff. This authorisation must be documented in the PMR 11B (or PRA template for older adult) prior to commencement of EGA.
- Staff escorting patients on EGA will have access to the ward mobile phone as a means of communication in the event of absconding/incident. If the phone is not available, EGA cannot occur.
- Patients on special observations, must maintain ratio of supporting workers to patient unless clearly documented. Any reason for additional workers must be considered when approving EGA. Security must be made aware of EGA for any patients with risks of aggression, including of duration and location of EGA. Carers and responsible adults are not to be included in this number of additional workers.

3.2 Unescorted Ground Access

UGA is not appropriate for patients with a high risk of absconding or who have current risk factors relating to suicide or aggression.

- The first period of UGA may only be authorised by medical staff, in consultation with the treating or Duty Consultant.
- Staff facilitating a patient's exit or access back into a ward will inform the Shift Co-ordinator. On return from leave this staff member must perform a minimum of a verbal check that patients do not have any contraband in their possession, and refer to [MHS Search of Person, Property, Environment and Seizure of Property Procedure](#). This must be done discreetly and with privacy and dignity in mind. Worker safety must be maintained at this time. The approach may range from asking the patient if they have any items in their possession to be placed in safe keeping or conducting a metal detector search for patients with a known history of returning contraband, refer to [MHS Alcohol, Illicit Substances and Non Prescribed Substances – Management of Procedure](#). The use of a metal detector should be noted in the patient's Treatment Support and Discharge Plan (TSDP) and leave sticker (non DMR sites)/ PMR 11B Mental Health Approved Leave Form (DMR sites, if form used). Two staff may be required to be present during checks.
- The delegated Nurse will record the leaving and returning times for UGA on both the PMR11 and the ward clinical observation checklist.
- Nursing staff must notify medical staff within 15 minutes should a patient not return from UGA.

3.3 Failure to Comply with Leave

If a voluntary patient leaves the hospital or refuses to return from leave, the treating team will consider the risks and determine whether the patient should be considered a missing person. Any other failure to comply with leave and conditions will form part of an individualised assessment for each patient and should be documented on the PMR 11B, leave sticker (non DMR sites)/PRA template for older adult services and TSDP.

3.4 Cultural Escorted Ground Access (CEGA), Cultural Unescorted Ground Access (CUGA) and Cultural Escorted Community Access (CECA)

For Aboriginal patients, cultural leave is essential as it allows them to fulfill important cultural obligations while receiving treatment. This time provides space for cultural responsibilities to be supported, thereby improving patient care and overall well-being. Cultural leave also enables the Aboriginal Liaison Officers (ALOs) to engage with patients in a culturally safe and appropriate manner, promoting a more holistic care approach.

The patients will need to be settled and low risk for the AMHLS to provide this type of support. A risk management plan will need to be explored and the ALO's supporting would need to comply with EGA Policy and Procedures

Cultural Intervention and activities for CEGA/CUGA/CECA

- **Creative Expression and Aboriginal Liaison partnership program (CECAT & AMHLS program)**
 - This partnership program fosters creativity and self-expression, empowering Aboriginal patients through therapeutic cultural activities designed to improve mental health and wellness.
- **Cultural Group Outing**
 - These groups are organised and planned by the AMHLS and Occupational Therapists (OT's)
- **NAIDOC Week**
 - Provides a platform to celebrate Aboriginal culture. Cultural leave allows patients to participate in local NAIDOC events, honouring their heritage and contributing to a sense of pride and belonging.
- **Sexual Assault Resource Centre (SARC)**
 - SARC counselling sessions are tailored specifically for Aboriginal consumers. These sessions offer culturally relevant support for patients dealing with trauma, grief, and other mental health challenges.
- **Hospital visits for family support**
 - In cases where a patient's family member is hospitalised or nearing the end of life, the AMHLS can provide crucial cultural support. This ensures the patient is supported in maintaining their cultural connection.
 - Sorry Business, funerals, might mean back to country during emotional times.
- **Sorry Business / Funerals**
 - In situations of grief, such as the loss of a loved one, patients may need to return to their country or attend ceremonies. Cultural leave facilitates these important rites of passage, supporting the healing process of grief and loss.
- **Cultural programs within the community**
 - External community programs and activities are integral to patient care. The AMHLS facilitates access to local cultural events, ensuring patients maintain their cultural ties and community involvement.

Benefits of Cultural Leave and Access

Providing Cultural CEGA and CUGA enables Aboriginal patients to fully engage in cultural programs, activities, and interventions without the concern of disrupting their treatment schedule or exceeding approved leave. This cultural leave approach encourages patients to actively

participate in cultural healing activities, which are crucial to their mental and emotional well-being. The use of CEGA and CUGA is more inclusive and efficient system of care that respects Aboriginal cultural practices.

Ultimately, this initiative helps the AMHLS better support the Aboriginal and Torres Strait Islander cohort within inpatient settings, offering culturally competent care that enhances patient outcomes.

3.5 Community – Other Leave

Community leave may be escorted where appropriate and may include supervision by National Disability Insurance Scheme (NDIS) workers, community staff or family members or others. Any persons supporting an inpatient on community leave should be aware of the relevant contacts should the patient abscond or refuse to return.

All staff must always carry a Personal Duress Alarm (PDA) and a charged mobile phone when in the community, including when transporting a consumer. There must be clear documentation - on the staff whereabouts board/file - of the staff members intended location, anticipated return time and name and contact of the consumer they will be visiting within the community.

If a patient becomes aggressive or violent during a community outing, the staff member(s) in attendance are to take reasonable steps, based on their assessment of the situation, to ensure the supervision and safety of the patient and any others present and avoid any actions that further place themselves or others at risk.

If patient, staff, or the public's safety is threatened contact Western Australian Police Force (WAPOL) for assistance or use duress alarm where available.

3.6 Escort Responsibilities

Students/Assistants in Nursing may not escort patients on EGA or community access without being accompanied by a Registered or Enrolled Nurse or another clinician. Newly qualified nurses/new staff will be expected to have been deemed competent or oriented to the expectations of the role.

There should always be consideration by the Shift Co-ordinator of the appropriateness of the escorting staff member.

Staff **must wear a PDA** and understand the relevant points of escalation and who to escalate to.

PDA's may not give an accurate location within the grounds, and as such, staff escorting within the grounds should have access to a mobile phone to summon assistance as required.

4. Definitions

Terms	Definitions
AWOL	Involuntary patients under the <i>Mental Health Act 2014</i> (MHA 2014) who are missing or suspected to be missing are defined as being absent without leave (AWOL)

Community access	Any area outside of the boundary of the hospital: • for Graylands Hospital and Selby Older Adult this is a specified authorised area including the grounds. • for Sir Charles Gairdner Hospital (SCGH) Mental Health Unit (MHU) itself is authorised and while the wider hospital provides a boundary any access outside of the unit would be classed as community access. • for Osborne Park Hospital, as an unauthorised facility, any leave from the unit is classed as community access
Escort	A person who accompanies/delivers a patient to an appointment and/or destination.
Escorted Ground Access (EGA)	Leave within a health service, usually within the hospital boundaries with supervision from a staff member/s:- • for Graylands Hospital this is a specified authorised area. • for SCGH this includes the wider hospital and while not authorised, provides a boundary within which patients should remain
Unescorted Ground Access (UGA)	Leave within a health service without supervision from a staff member: • for Graylands Hospital and Selby Older Adult this is a specified authorised area. • For SCGH MHU and Osborne Park Hospital Older Adult, while the wider hospital provides a boundary within which patients should remain, it is technically community access and should be treated as such.

5. Compliance, monitoring and evaluation

NMHS MH services will monitor performance with the following performance indicators:

- 100% of patients will have a risk assessment prior to EGA or UGA being granted.
- 100% of patients leave will be documented on the Form 11B where utilised.
- Ongoing review of clinical incidents attached to use of EGA and UGA.
- Ongoing review of WHS incidents during EGA/UGA and subsequent contraband related incidents (e.g. smoking in wards).

This procedure will be evaluated as required to determine its currency, relevance, effectiveness, efficiency, and sustainability. At a minimum, it will be reviewed every three years using a risk management approach in accordance with the [NMHS Policy Governance Framework](#).

Refer to [NMHS Policy Document Glossary of Terms](#) for any other frequently used terms

6. Related Documents

Legislation	• Mental Health Act 2014 (MHA 2014) • Work Health and Safety Act 2020
WA Health mandatory policies	• MP 0012/16 Mental Health Consumers Who Are Missing or Absent Without Leave Policy • MP 00122/19 Clinical Incident Management Policy • Clinical Incident Management Guideline 2019. •
NMHS Policy documents	• NMHS Clinical Incident Management Policy • NMHS Hazard/Incident Reporting and Investigation Policy • NMHS Preventing and Managing Workplace Violence and Aggression (WVA) • NMHS Open Disclosure Policy • NMHS Duress Alarm Policy
Standards	• Clinical Governance (Standard 1) • Partnering with Consumers (Standard 2)
Office of the Chief Psychiatrist	• The Chief Psychiatrist's Standards for Authorised Hospitals
MHS Policy documents	• MHS Clinical Risk Assessment and Management Policy • MHS Search of Person, Property, Environment and Seizure of Property Procedure • MHS Alcohol , Illicit Substances and Non-Prescribed Substances – Management of Procedure

7. Document Control

Document Version Control			
Date	Version	Author	Notes
09/02/2024	V1.0	Medical Co Director Adult Inpatient MH	Initial endorsement, provides guidance for staff escorting patients in the community and/or hospital grounds.
19/11/2025	V2.0	Operational Co Director Adult Inpatient MH	Review of process relating to leave form, PRA information and leave sticker, added information in relation to Cultural Escorted Ground Access , Cultura Unescorted Ground Access and Cultural Escorted Community Access.

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

Copyright to this material is vested in the State of Western Australia unless otherwise indicated. Apart from any fair dealing for the purposes of private study, research, criticism, or review, as permitted under the provisions of the *Copyright Act 1968*, no part may be reproduced or re-used for any purposes whatsoever without written permission of the State of Western Australia.

Appendix 1: Leave Sticker – sample only

LEAVE STICKER

To be completed **AFTER** completion of PRA

Patient name:

MHA Status: Voluntary ☐ Involuntary ☐

Type of Leave	Tick if applicable	Conditions of leave (e.g. length of leave, if escorted who with)
No Leave		
Escorted Ground Access		
Unescorted Ground Access		
Escorted Leave		
Unescorted Leave		
Leave Revoked		

Action required if absconds:

Date:

Time:

Signed (Psych Reg / Consultant):

Complete new sticker for changes / updates to leave

G253 HCHGRLAB0253 08/23

G253 HCHGRLAB0253.indd 1

21/8/23 1:11 pm

Appendix 2: Mental Health Approved Leave Form (1 of 2) – sample only

0262
08/24

DO NOT WRITE IN MARGIN



EMR310340

NORTH METROPOLITAN AREA HEALTH SERVICE
MENTAL HEALTH

**MENTAL HEALTH
APPROVED LEAVE FORM**

WARD _____

PSYCHIATRIST / REGISTRAR _____

**LEAVE ORDERS TO BE
REVIEWED WEEKLY**

Please use I.D. label or block print

SURNAME _____ UMRN _____

GIVEN NAMES _____ DOB _____ GENDER _____

ADDRESS _____ POSTCODE _____

TELEPHONE _____

Business Rules

- All leave must be granted on the approval of the treating psychiatrist
- Leave must be documented in progress notes including a **Risk assessment** and **Actions required if patient absconds**
- Involuntary patients require a form 7A for overnight leave
- Voluntary patients must have start & finish date / time documented for overnight leave
- Cross out 'old' approvals and ensure new approval is signed

Date & Time	MHA Status	Type of Leave	Duration of Leave	Frequency of Leave	Conditions of leave	Actions if absconds	Psych Reg / Consultant signature and print
00/00/00 09:00	Invol	EGA	30/60	TDS	Escorted by nursing staff or family. At nurses discretion.	AWL and UDS on return	
00/00/00 09:00	Invol	ONL	2 nights	Once Only	To mother house only. Leave 15/4 1000hrs, return 17/4 by 1800hrs. UDS and breathalyser on return.	AWL, UDS and breathalyser on return	

Page 1 of 2

PMR11B MENTAL HEALTH APPROVED LEAVE FORM

Appendix 2: Mental Health Approved Leave Form (1 of 2) – sample only

[illegible]